PEDIATRIC SURGERY · COMPLETE QUESTION BANK

Pediatric Surgical Nursing All Lectures Question Bank

جراحة الأطفال — مجمّع أسئلة المادة كلها

5	82	30	54
Lectures	Past exam questions	NCLEX-style	Flashcards

#	Lectures included in this file · المحاضرات اللي في الملف	Past Qs	NCLEX	Cards
1	Pediatric Surgical Care الرعاية الجراحية للأطفال	10	6	12
2	Common Pediatric Surgical Conditions (Hernia, Hydrocele & UDT) الحالات الجراحية الشائعة (الفتق والقيلة المائية والخصية المعلقة)	21	6	12
3	CHPS & Tracheo-Oesophageal Fistula ضيق البواب الخلقي والناسور المريئي الرغامي	20	6	12
4	Intussusception الانغلاف المعوي	5	6	7
5	Anorectal Malformations تشوهات الشرج والمستقيم	26	6	11

المصادر:

سؤال فاينال (صورة من الجروب) · فاينال (مراجعة أسئلة) · ميدترم 2025 · ميدترم السنة اللي فاتت (Microsoft Forms) · كويز 1 – 2023/2024 (شعبة 4) (أطفال نظري) · أسئلة سابقة

ترقيم الأسئلة: رقم المحاضرة ثم رقم السؤال (مثلاً 4.12). أسئلة NCLEX عليها حرف N. الإجابات كلها في آخر الملف.

اللَّهُمَّ لا سَهْلَ إِلَّا ما جَعَلْتَهُ سَهْلًا، وأنتَ تَجْعَلُ الحَزْنَ إذا شِئْتَ سَهْلًا — صلِّ على النبي ﷺ

LECTURE 1

Pediatric Surgical Care

الرعاية الجراحية للأطفال

Part A · Past Exam Questions

أسئلة الدفعات السابقة

1.1 Bilious vomiting in neonates: — ميدترم السنة اللي فاتت (Microsoft Forms)

a. Is an emergency
b. Surgical consultation is a must
c. It may be life threatening
d. All of the above

1.2 You should teach the parents, when needed, all of the following EXCEPT: — ميدترم السنة اللي فاتت (Microsoft Forms)

a. Colostomy care
b. Gastrostomy feeds
c. Esophagostomy care
d. Intravenous antibiotics

1.3 Complete: A complication of blood transfusion is ..... — فاينال (مراجعة أسئلة)

.....

1.4 Complete: Complications of laparoscopic surgery are ..... — فاينال (مراجعة أسئلة)

.....

1.5 Complete: Signs of severe hypothermia ..... — فاينال (مراجعة أسئلة)

.....

1.6 Complete: Hypothermia in the neonate may ..... — فاينال (مراجعة أسئلة)

.....

1.7 Central venous access needs special handling and frequent dressing. (True / False) — فاينال (مراجعة أسئلة)

a. True
b. False

1.8 You should ask the pediatric surgeon before you start feeds in gastrostomy. (True / False) — فاينال (مراجعة أسئلة)

a. True
b. False

1.9 You should not teach parents colostomy care. (True / False) — فاينال (مراجعة أسئلة)

a. True
b. False

1.10 Bilious vomiting can be an emergency. (True / False) — أسئلة سابقة

a. True
b. False

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

1.N1 **Which pain scale is suitable for a 2-year-old after surgery?** NCLEX

a. Numeric 0–10 scale
b. FLACC scale
c. Visual analogue scale
d. McGill questionnaire

1.N2 **Parents refuse a life-saving emergency operation for their child. The surgeon proceeds. Which principle justifies this?** NCLEX

a. Autonomy of parents
b. Best interest of the child (beneficence)
c. Fidelity
d. Veracity

1.N3 **Select ALL components of preoperative care of a child.** SELECT ALL

a. Hydration and NPO status
b. Age-appropriate preparation to reduce anxiety
c. Informed consent
d. Monitoring vital signs
e. Giving a heavy meal before anesthesia

1.N4 **Why are neonates at special risk of hypothermia in the operating room?** NCLEX

a. Thick subcutaneous fat
b. Large surface area to weight ratio and little fat
c. They shiver efficiently
d. Low metabolic rate

1.N5 **A child receiving multiple blood transfusions is at risk of:** NCLEX

a. Hypokalemia
b. Hemosiderosis (iron overload)
c. Hypoglycemia
d. Polycythemia

1.N6 **A neonate with bilious vomiting is admitted. The nurse's priority is:** NCLEX

a. Give oral fluids
b. Keep NPO, insert NGT, start IV fluids and notify the surgeon
c. Wait and observe
d. Give antiemetic and feed

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Pediatric surgery differences	Anatomical, physiological and psychological.	**Informed consent**	From parents/guardian; child assent according to age.
Emergency without consent	Allowed for child's best interest.	**Pre-op care**	NPO, hydration, emotional support, pain management, monitoring.
Post-op assessment	Vital signs, pain (FLACC/Wong-Baker), wound, neurologic status.	**Bilious vomiting**	Surgical emergency in neonate until proven otherwise.

Hypothermia risk	Large surface area, little fat, exposure in OR.
Hypothermia management	Warm environment, remove wet clothes, blankets, warm fluids, warmed O2.
Blood transfusion complications	Hemolytic reaction, overload, hyperkalemia, embolism, thrombophlebitis, hemosiderosis.
Laparoscopy pros	Minimally invasive, faster recovery, cosmetic, less blood loss.
Central venous line	Special handling and frequent sterile dressing.
Gastrostomy feeds	Start after surgeon's order; teach parents.

LECTURE 2

Common Pediatric Surgical Conditions (Hernia, Hydrocele & UDT)

الحالات الجراحية الشائعة (الفتق والقيلة المائية والخصية المعلقة)

Part A · Past Exam Questions

أسئلة الدفعات السابقة

2.1 **Strangulation in inguinal hernia:** ميدترم السنة اللي فاتت (Microsoft Forms)

a. May cause death
b. Is due to delay in diagnosis
c. Is due to delay in management
d. All of the above

2.2 **Glans injury is a dangerous injury during ..... surgery.** ميدترم السنة اللي فاتت (Microsoft Forms)

a. Circumcision
b. Cleft palate
c. Tongue tie
d. Cleft lip

2.3 **In cases of neonatal inguinal hernia, we wait for the age of ..... to do surgery.** ميدترم السنة اللي فاتت (Microsoft Forms)

a. 1 year
b. 2 years
c. 3 years
d. None of the above — once diagnosed, do surgery as soon as possible

2.4 **At what age is surgical correction recommended for undescended testes?** ميدترم 2025

a. 1 month
b. 6 months to 1 year
c. 5 years
d. Adolescence

2.5 **Most common site for undescended testes?** ميدترم 2025

a. Abdominal cavity
b. Inguinal canal
c. Scrotal neck
d. Perineum

2.6 **Congenital hydrocele is caused by:** ميدترم 2025

a. Trauma
b. Infection
c. Incomplete closure of processus vaginalis
d. Tumor

2.7 **What is the main clinical feature of hydrocele?** ميدترم 2025

a. Hard, tender scrotal swelling
b. Soft, non-tender scrotal swelling that transilluminates
c. Scrotal redness
d. Fever

2.8 The treatment for the most common congenital hydrocele is: ميدترم 2025

a. Immediate surgery
b. Observation until the age of 2 years
c. Antibiotics
d. Diuretics

2.9 What is the most common type of hernia in children? ميدترم 2025

a. Umbilical hernia
b. Inguinal hernia
c. Femoral hernia
d. Epigastric hernia

2.10 Which of the following is a sign of an incarcerated inguinal hernia? ميدترم 2025

a. Reducible, painless swelling
b. Non-reducible, tender swelling
c. Swelling only during crying
d. No swelling

2.11 Definitive treatment of inguinal hernia in a child: ميدترم 2025

a. Manual reduction
b. Herniotomy/herniorrhaphy (surgical repair)
c. Observation
d. Use of truss

2.12 Which of the following is NOT associated with untreated cryptorchidism (undescended testis)? ميدترم 2025

a. Infertility
b. Testicular torsion
c. Appendicitis
d. Testicular cancer

2.13 Complete: Indications of circumcision are ..... فاينال (مراجعة أسئلة)

.....

2.14 Complete: Common risks after circumcision are ..... فاينال (مراجعة أسئلة)

.....

2.15 In case of inguinal hernia, wait for surgery until the child reaches 3 years. (True / False) فاينال (مراجعة أسئلة)

a. True
b. False

2.16 Umbilical hernia is an emergency condition. (True / False) فاينال (مراجعة أسئلة)

a. True
b. False

2.17 In case of unpalpable undescended testis, perform MRI. (True / False) فاينال (مراجعة أسئلة)

a. True
b. False

2.18 Glans injury is most common in circumcision. (True / False) فاينال (مراجعة أسئلة)

a. True
b. False

2.19 Inguinal hernia in females does not need surgery. (True / False) أسئلة سابقة

a. True
b. False

2.20 Inguinal hernia may be treated by antibiotics or needs MRI. (True / False) أسئلة سابقة

a. True **b.** False

2.21 Complete: Inguinal hernia surgery should be done early to prevent ..... أسئلة سابقة

.....

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

2.N1 A 3-month-old has a groin swelling that increases with crying and reduces at rest. The nurse anticipates: NCLEX

a. Observation until 3 years
b. Elective herniotomy soon after diagnosis
c. Antibiotics
d. Truss application

2.N2 Which finding indicates an incarcerated inguinal hernia? NCLEX

a. Soft, reducible swelling
b. Tender, irreducible swelling with vomiting
c. Swelling that transilluminates
d. Swelling only when crying

2.N3 Select ALL features of a congenital communicating hydrocele. SELECT ALL

a. Due to patent processus vaginalis **b.** Soft, painless scrotal swelling
c. Transilluminates **d.** Usually needs emergency surgery
e. Many resolve by 1–2 years

2.N4 Orchiopexy for undescended testis is ideally performed at: NCLEX

a. Birth **b.** 6–12 months (before 1–2 years)
c. 5 years **d.** Puberty

2.N5 Complications of untreated cryptorchidism include all EXCEPT: NCLEX

a. Infertility **b.** Testicular torsion
c. Testicular cancer **d.** Appendicitis

2.N6 After circumcision, the nurse teaches parents to watch mainly for: NCLEX

a. Bleeding and signs of infection **b.** Constipation
c. Hearing problems **d.** Rash on the face

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Inguinal hernia	Most common hernia in children; patent processus vaginalis.	**Inguinal hernia timing**	Operate once diagnosed (no waiting).

Incarceration	Irreducible tender swelling — emergency.
Strangulation	Compromised blood supply; may cause death; due to delayed diagnosis/management.
Hernia in females	Needs surgery — may contain the ovary.
Hydrocele	Fluid in tunica vaginalis; soft, painless, transilluminates.
Hydrocele treatment	Observe until ~2 years; surgery if persists/communicating.
Undescended testis	Most common site: inguinal canal.
Orchiopexy age	6–12 months (before 2 years).
UDT complications	Infertility, torsion, malignancy, hernia, psychological.
Unpalpable testis	Laparoscopy (not MRI).
Circumcision risks	Bleeding, infection, pain; glans injury is serious.

LECTURE 3

CHPS & Tracheo-Oesophageal Fistula

ضيق البواب الخلقي والناسور المريئي الرغامي

Part A · Past Exam Questions

أسئلة الدفعات السابقة

3.1 In cases of TOF/OA (tracheo-oesophageal fistula / oesophageal atresia): (Microsoft Forms) ميدترم السنة اللي فاتت

a. It's an emergency surgery to ligate the fistula in case of associated imperforate anus
b. It can be done by thoracoscopy
c. We may do feeding gastrostomy if anastomosis failed
d. All of the above

3.2 In case of IHPS (infantile hypertrophic pyloric stenosis): (Microsoft Forms) ميدترم السنة اللي فاتت

a. It needs pyloromyotomy
b. Can be done by thoracoscopy
c. Give the baby blood always
d. Vomiting is always bilious

3.3 The patient with esophageal atresia with or without fistula may present with all EXCEPT: (Microsoft Forms) ميدترم السنة اللي فاتت

a. Excessive salivation
b. Gasless abdomen in X-ray in type A esophageal atresia
c. Bilious vomiting
d. Chest complications

3.4 You receive a baby after birth; how do you suspect esophageal atresia? (Microsoft Forms) ميدترم السنة اللي فاتت

a. Visible abdominal pain
b. Failure to pass meconium
c. Failure to pass Ryle (NGT)
d. Vomiting

3.5 Classic symptom of pyloric stenosis in the infant is: ميدترم 2025

a. Projectile bilious vomiting
b. Projectile non-bilious vomiting
c. Bloody vomiting
d. Vomiting with fever

3.6 Which electrolyte abnormality is commonly associated with pyloric stenosis? ميدترم 2025

a. Hyperchloremic metabolic acidosis
b. Hypochloremic metabolic alkalosis
c. Hyperkalemia
d. Hyponatremia

3.7 Significant treatment of pyloric stenosis: ميدترم 2025

a. Antibiotic therapy
b. Pyloromyotomy
c. Nasogastric decompression
d. Fluid restriction

3.8 Which clinical finding is most suggestive of esophageal atresia in a newborn? ميدترم 2025

a. Excessive oral saliva secretion
b. Diarrhea
c. Hypotonia
d. Seizures

3.9 Esophageal atresia is most associated with which condition? — ميدترم 2025

a. Down syndrome
b. VACTERL association
c. Turner syndrome
d. Marfan syndrome

3.10 Which diagnostic test confirms esophageal atresia? — ميدترم 2025

a. Blood test
b. Passage of a nasogastric tube and X-ray
c. Urinalysis
d. Echocardiography

3.11 In all cases of tracheoesophageal fistula we perform tracheostomy and gastrostomy. (True / False) — فاينال (مراجعة أسئلة)

a. True
b. False

3.12 CHPS commonly causes bilious vomiting. (True / False) — فاينال (مراجعة أسئلة)

a. True
b. False

3.13 A child with esophageal atresia is given oral antibiotics. (True / False) — فاينال (مراجعة أسئلة)

a. True
b. False

3.14 Treatment of esophageal atresia is surgery after preparation. (True / False) — فاينال (مراجعة أسئلة)

a. True
b. False

3.15 Complete: Tracheoesophageal fistula with imperforate anus is managed as ..... — أسئلة سابقة

.....

3.16 Complete: The type of surgery in tracheoesophageal fistula can be ..... — أسئلة سابقة

.....

3.17 Complete: If anastomosis fails in esophageal atresia we do ..... — أسئلة سابقة

.....

3.18 Complete: The surgery of hypertrophic pyloric stenosis is ..... — أسئلة سابقة

.....

3.19 In tracheoesophageal fistula, vomiting may be bilious. (True / False) — أسئلة سابقة

a. True
b. False

3.20 Tracheoesophageal fistula may cause intestinal obstruction. (True / False) — أسئلة سابقة

a. True
b. False

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

3.N1 A 5-week-old boy has projectile non-bilious vomiting after every feed and is hungry after vomiting. The nurse suspects: NCLEX

a. Gastroesophageal reflux
b. Hypertrophic pyloric stenosis
c. Duodenal atresia
d. Intussusception

3.N2 Which laboratory pattern is expected in pyloric stenosis? NCLEX

a. Hyperchloremic acidosis
b. Hypochloremic, hypokalemic metabolic alkalosis
c. Hyperkalemia
d. Metabolic acidosis with high anion gap

3.N3 Select ALL pre-operative priorities for an infant with pyloric stenosis. SELECT ALL

a. Correct dehydration and electrolytes with IV fluids
b. Keep NPO with NG decompression as ordered
c. Monitor intake, output and weight
d. Start large oral feeds immediately
e. Schedule pyloromyotomy after correction

3.N4 The first sign that should make the nurse suspect esophageal atresia in a newborn is: NCLEX

a. Bilious vomiting
b. Excessive frothy salivation and choking with the first feed
c. Abdominal distension only
d. Failure to pass meconium

3.N5 Esophageal atresia is commonly associated with which group of anomalies? NCLEX

a. VACTERL
b. TORCH
c. CHARGE only
d. Pierre Robin

3.N6 After TEF repair, the nurse notes the chest tube. Which action is correct? NCLEX

a. Clamp it routinely
b. Keep the drainage system below chest level and monitor drainage
c. Milk it vigorously every hour
d. Remove it if bubbling stops

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

CHPS	Hypertrophy of pyloric muscle → gastric outlet obstruction; 3–6 weeks; boys, first-born.	**CHPS signs**	Projectile non-bilious vomiting, hungry baby, olive mass, visible peristalsis.
CHPS labs	Hypochloremic hypokalemic metabolic alkalosis.	**CHPS surgery**	Ramstedt pyloromyotomy after fluid/electrolyte correction (not emergency).

Post-pyloromyotomy feeding	Clear fluids 4–6 h, then gradual feeds; burp; right side/Fowler's.
OA signs	Excess salivation, choking, cyanosis, NG tube can't pass.
OA pre-op	NPO, suction of upper pouch, head up, IV fluids, warm.
TOF + imperforate anus	Emergency ligation of fistula.

OA/TOF	Esophageal atresia ± tracheoesophageal fistula; commonest type C (distal TEF).
Type A OA	Pure atresia — gasless abdomen on X-ray.
OA surgery	Thoracotomy/thoracoscopy repair; gastrostomy if anastomosis fails.
VACTERL	Vertebral, anorectal, cardiac, TE, renal, limb.

LECTURE 4

Intussusception

الانغلاف المعوي

Part A · Past Exam Questions

أسئلة الدفعات السابقة

4.1 **Intussusception:** ميدترم السنة اللي فاتت (Microsoft Forms)

a. Always needs surgery
b. Pneumatic reduction may solve the problem
c. You can wait for 8 hours
d. The treatment is medical

4.2 **Air enema is both diagnostic and therapeutic in intussusception. (True / False)** سؤال فاينال (صورة من الجروب)

a. True **b.** False

4.3 **In intussusception, surgery is always needed. (True / False)** أسئلة سابقة

a. True **b.** False

4.4 **Which clinical manifestations would the nurse expect to observe while caring for the child diagnosed with intussusception?** تكرر في 2 مصادر أسئلة سابقة

a. Currant jelly stool **b.** Foul-smelling stool
c. Dark brown stool **d.** Watery stool

4.5 **Which clinical manifestations would the nurse expect to observe while caring for the child diagnosed with intussusception?** كويز 1 – 2023/2024 (شعبة 4) (أطفال نظري)

تكرر في 2 مصادر

a. Currant jelly stool **b.** Foul-smelling stool
c. Dark brown stool **d.** Watery stool

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

4.N1 **A 9-month-old has sudden colicky pain with drawing up of legs, vomiting and red currant-jelly stools. The likely diagnosis is:** NCLEX

a. Pyloric stenosis **b.** Intussusception
c. Hirschsprung disease **d.** Appendicitis

4.N2 **First-line management of uncomplicated intussusception is:** NCLEX

a. Immediate laparotomy
b. Air (pneumatic) or hydrostatic enema reduction
c. Antibiotics only
d. Laxatives

4.N3 **Select ALL signs that a reduced intussusception has resolved.** SELECT ALL

a. Passage of normal brown stool
b. Disappearance of the abdominal mass
c. Child becomes calm and pain-free
d. Return of currant-jelly stool
e. Increasing distension

4.N4 **The most common type of intussusception in children is:** NCLEX

a. Ileocolic
b. Colocolic
c. Jejunojejunal
d. Gastroduodenal

4.N5 **The diagnostic imaging of choice for intussusception is:** NCLEX

a. Abdominal ultrasound (target sign)
b. Barium swallow
c. CT head
d. Chest X-ray

4.N6 **Peak age for intussusception is:** NCLEX

a. Neonatal period
b. 6 months to 2 years
c. School age
d. Adolescence

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Intussusception	Telescoping of bowel (ileocolic); 6 months–2 years.
Intussusception triad	Colicky pain, sausage mass, currant-jelly stool.
Intussusception diagnosis	Ultrasound (target sign).
Intussusception treatment	Air/hydrostatic enema (diagnostic + therapeutic); surgery if fails.
Mass	Sausage-shaped RUQ mass; empty RLQ (Dance sign).
Red flags	Peritonitis, perforation, shock → surgery.
After reduction	Watch for normal stool; recurrence risk.

LECTURE 5

Anorectal Malformations

تشوهات الشرج والمستقيم

Part A · Past Exam Questions

أسئلة الدفعات السابقة

5.1 All of the following are preoperative preparations of neonatal intestinal obstruction EXCEPT: (Microsoft Forms) ميدترم السنة اللي فاتت

a. Intravenous fluids
b. CBC, serum electrolytes and coagulation profile
c. Ryle/NGT
d. Oral feeding

5.2 Meconium ileus is a cause of bowel obstruction in: (Microsoft Forms) ميدترم السنة اللي فاتت

a. Neonates
b. Toddlers
c. Preschool
d. School age

5.3 In cases of high imperforate anus we do: (Microsoft Forms) ميدترم السنة اللي فاتت

a. Anoplasty
b. Colostomy, anoplasty
c. Colostomy, anoplasty, closure of colostomy
d. Conservative management

5.4 Which of the following is an essential nursing care in all babies with intestinal obstruction? (Microsoft Forms) ميدترم السنة اللي فاتت

a. Endotracheal tube
b. Intravenous line
c. Clear oral fluids
d. Enema

5.5 Presentation of intestinal obstruction includes the following EXCEPT: (Microsoft Forms) ميدترم السنة اللي فاتت

a. Dehydration
b. Low urine output
c. Hyperactive baby
d. Vomiting

5.6 Causes of neonatal intestinal obstruction include all of the following EXCEPT: (Microsoft Forms) ميدترم السنة اللي فاتت

a. Jejunal atresia
b. Meconium ileus
c. Tracheo-esophageal fistula
d. Hirschsprung's disease

5.7 In intestinal obstruction, what do you insert to protect from aspiration? (Microsoft Forms) ميدترم السنة اللي فاتت

a. NGT/Ryle
b. Urine catheter
c. Umbilical catheter
d. Central line

5.8 Which of the following is a common sign of anorectal malformation in a newborn? ميدترم 2025

a. Excessive urination
b. No meconium passage and abdominal distention
c. Jaundice
d. Cyanosis

5.9 What is the imaging study special for evaluation of anorectal malformation? ميدترم 2025

a. MRI
b. Cross-table lateral X-ray
c. CT scan
d. Ultrasound

5.10 What is the most common anorectal malformation in males? ميدترم 2025

a. Rectovaginal fistula
b. Rectourethral fistula
c. Rectoperineal fistula
d. Rectovesical fistula

5.11 Which sign is the most consistent with duodenal atresia? ميدترم 2025

a. Triple bubble sign
b. Double bubble sign
c. Air-fluid levels
d. Pneumoperitoneum

5.12 What is the first management of neonatal intestinal atresia? ميدترم 2025

a. Immediate surgical intervention
b. Antibiotic therapy
c. Nasogastric decompression
d. Fluid restriction

5.13 Complete: Causes of intestinal obstruction in the neonate ..... فاينال (مراجعة أسئلة)

.....

5.14 Complete: Rectal biopsy is used to diagnose ..... فاينال (مراجعة أسئلة)

.....

5.15 Complete: Double bubble sign on X-ray is seen in ..... فاينال (مراجعة أسئلة)

.....

5.16 In all cases of imperforate anus we perform colostomy. (True / False) فاينال (مراجعة أسئلة)

a. True
b. False

5.17 Hirschsprung's disease affects the colon of the neonate and results in midgut volvulus. (True / False) فاينال (مراجعة أسئلة)

a. True
b. False

5.18 Surgical treatment of Hirschsprung's disease is removal of the aganglionic segment. (True / False) فاينال (مراجعة أسئلة)

a. True
b. False

5.19 Meconium ileus causes bowel obstruction in neonates. (True / False) فاينال (مراجعة أسئلة)

a. True
b. False

5.20 In imperforate anus, wait until the infant reaches 30 days to perform surgery. (True / False) فاينال (مراجعة أسئلة)

a. True **b.** False

5.21 Vomiting and failure to pass stool are signs of intestinal obstruction. (True / False) فاينال (مراجعة أسئلة)

a. True **b.** False

5.22 Dehydration can be present in neonatal intestinal obstruction. (True / False) فاينال (مراجعة أسئلة)

a. True **b.** False

5.23 Complete: The operation for low imperforate anus is ..... أسئلة سابقة

.....

5.24 Complete: The operation for high imperforate anus is ..... أسئلة سابقة

.....

5.25 Complete: If a newborn does not pass meconium, suspect ..... أسئلة سابقة

.....

5.26 Complete: A child has intestinal obstruction; to prevent aspiration you ..... أسئلة سابقة

.....

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

5.N1 A newborn has no anal opening. The surgical plan for a HIGH anomaly is: NCLEX

a. Single anoplasty
b. Colostomy, then anoplasty (PSARP), then closure of colostomy
c. Observation
d. Dilatation only

5.N2 The nurse should first suspect imperforate anus when: NCLEX

a. The baby has jaundice
b. The first rectal temperature cannot be taken or no meconium passes in 24 h
c. The baby vomits once
d. The baby is cyanosed

5.N3 Colostomy care teaching for parents includes: NCLEX

a. Apply tight dressing over the stoma
b. Clean skin around stoma, use a well-fitting bag and watch stoma color
c. Avoid emptying the bag
d. Use alcohol on the stoma

5.N4 A cross-table lateral X-ray for a newborn with ARM is best done after: NCLEX

a. 1 hour
b. 24 hours
c. 1 week
d. 1 month

5.N5 The most common anorectal malformation in females is: NCLEX

a. Rectovestibular fistula
b. Rectourethral fistula
c. Cloaca only
d. Rectovesical fistula

5.N6 ARM is part of which association? NCLEX

a. VACTERL
b. TORCH
c. Down syndrome only
d. Turner

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Anorectal malformation	Spectrum of imperforate anus; low vs high.	**ARM imaging**	Cross-table lateral X-ray (invertogram) after 24 h.
Commonest ARM in males	Rectourethral fistula.	**Low ARM surgery**	Anoplasty.
High ARM surgery	Colostomy → PSARP/anoplasty → closure of colostomy.	**Neonatal obstruction causes**	Duodenal/jejunal atresia, ARM, Hirschsprung, meconium ileus, malrotation.
Double bubble	Duodenal atresia.	**Obstruction nursing**	NPO, NGT to prevent aspiration, IV line, fluids & electrolytes.
Females	Rectovestibular fistula commonest.	**Wangensteen/invert**	Lateral X-ray after 24 h.
Post-PSARP	Anal dilatation program.		

Answer Key — All Lectures

الإجابات

1.1. d — All of the above

1.2. d — Intravenous antibiotics

1.3. ✎ — Hyperkalemia (also hemolytic reaction, circulatory overload, embolism, thrombophlebitis, hemosiderosis)

1.4. ✎ — Vessel injury, organ injury, bowel injury

1.5. ✎ — Loss of shivering, hypotension, hypoactivity/coma, cardiac arrest

1.6. ✎ — Cause death

1.7. a — True

1.8. a — True

1.9. b — False

1.10. a — True

1.N1. b — FLACC scale
FLACC (Face, Legs, Activity, Cry, Consolability) suits preverbal children; Wong-Baker FACES for ~3 years and older.

1.N2. b — Best interest of the child (beneficence)
In emergencies the child's best interest allows treatment without consent.

1.N3. a, b, c, d — Hydration and NPO status; Age-appropriate preparation to reduce anxiety; Informed consent; Monitoring vital signs
Preoperative care: preparation (NPO, hydration), emotional support, consent, pain management and monitoring.

1.N4. b — Large surface area to weight ratio and little fat
Large surface area, thin skin and limited fat increase heat loss; shivering is absent in neonates.

1.N5. b — Hemosiderosis (iron overload)
Repeated transfusions lead to iron overload; hyperkalemia is another complication.

1.N6. b — Keep NPO, insert NGT, start IV fluids and notify the surgeon
Bilious vomiting in a neonate is a surgical emergency (e.g., malrotation/volvulus) until proven otherwise.

2.1. d — All of the above

2.2. a — Circumcision

2.3. d — None of the above — once diagnosed, do surgery as soon as possible

2.4. b — 6 months to 1 year

2.5. b — Inguinal canal

2.6. c — Incomplete closure of processus vaginalis

2.7. b — Soft, non-tender scrotal swelling that transilluminates

2.8. b — Observation until the age of 2 years

2.9. b — Inguinal hernia

2.10. b — Non-reducible, tender swelling

2.11. b — Herniotomy/herniorrhaphy (surgical repair)

2.12. c — Appendicitis

2.13. ✎ — Phimosis, paraphimosis, recurrent UTI

2.14. ✎ — Bleeding, infection, pain

2.15. b — False

2.16. b — False

2.17. b — False

2.18. b — False
مع إن ميدترم، (الأشبع بعد الطهارة النزيف والعدوى) False الملف كاتب إصابة خطيرة في الطهارة — glans injury السنة اللي فاتت قال إن خطيرة ≠ الأشبع.

2.19. b — False
False — لازم جراحة لأن الفتق ممكن يكون فيه المبيض.

2.20. b — False

2.21. ✎ — Strangulation

2.N1. b — Elective herniotomy soon after diagnosis
Inguinal hernia does not resolve spontaneously; surgery is done once diagnosed to prevent incarceration.

2.N2. b — Tender, irreducible swelling with vomiting
Irreducible tender swelling with vomiting/irritability means incarceration — an emergency.

2.N3. a, b, c, e — Due to patent processus vaginalis; Soft, painless scrotal swelling; Transilluminates; Many resolve by 1–2 years
Most congenital hydroceles resolve spontaneously; surgery if persistent after ~2 years.

2.N4. b — 6–12 months (before 1–2 years)
Early orchiopexy protects fertility and allows malignancy surveillance.

2.N5. d — Appendicitis
Appendicitis is unrelated.

2.N6. a — Bleeding and signs of infection
Bleeding, infection and pain are the common risks after circumcision.

3.1. d — All of the above
سؤال 7 من الامتحان ده مش موجود في الصور. أسئلة diaphragmatic hernia وlymphangioma وNEC اتشالت لأنها مش في قائمة المحاضرات.

3.2. a — It needs pyloromyotomy

3.3. c — Bilious vomiting

3.4. c — Failure to pass Ryle (NGT)

3.5. b — Projectile non-bilious vomiting
ملف ميدترم 2025 من غير إجابات (الطالب كاتب «حله لوحدك») — كل إجابات الامتحان ده مقترحة ومراجَعة على الملخص.

3.6. b — Hypochloremic metabolic alkalosis

3.7. b — Pyloromyotomy

3.8. a — Excessive oral saliva secretion

3.9. b — VACTERL association

3.10. b — Passage of a nasogastric tube and X-ray

3.11. b — False

3.12. b — False

3.13. b — False

3.14. a — True

3.15. ✎ — An emergency

3.16. ✎ — Thoracoscopy

3.17. ✎ — Gastrostomy

3.18. ✎ — Pyloromyotomy

3.19. b — False

3.20. b — False

3.N1. b — Hypertrophic pyloric stenosis
CHPS classically appears at 3–6 weeks with projectile non-bilious vomiting and a hungry infant.

3.N2. b — Hypochloremic, hypokalemic metabolic alkalosis
Loss of gastric HCl through vomiting produces hypochloremic hypokalemic metabolic alkalosis.

3.N3. a, b, c, e — Correct dehydration and electrolytes with IV fluids; Keep NPO with NG decompression as ordered; Monitor intake, output and weight; Schedule pyloromyotomy after correction
Pyloromyotomy is never an emergency; fluids and electrolytes are corrected first.

3.N4. b — Excessive frothy salivation and choking with the first feed
Excessive saliva, coughing, choking and cyanosis with feeding; the NG tube cannot be passed.

3.N5. a — VACTERL
VACTERL: Vertebral, Anorectal, Cardiac, Tracheo-Esophageal, Renal, Limb anomalies.

3.N6. b — Keep the drainage system below chest level and monitor drainage
Keep the system closed and below the chest; monitor amount and character of drainage.

4.1. b — Pneumatic reduction may solve the problem

4.2. a — True
تعليق الصورة على الجروب قال إن الإجابة الرسمية في الفورم True. الـ barium enema كمان diagnostic وtherapeutic.)

4.3. b — False

4.4. a — Currant jelly stool

4.5. a — Currant jelly stool

4.N1. b — Intussusception
Classic triad: colicky pain, sausage-shaped mass and currant-jelly stool.

4.N2. b — Air (pneumatic) or hydrostatic enema reduction
Enema reduction is both diagnostic and therapeutic; surgery if it fails or peritonitis is present.

4.N3. a, b, c — Passage of normal brown stool; Disappearance of the abdominal mass; Child becomes calm and pain-free
Passage of normal stool and relief of pain indicate successful reduction.

4.N4. a — Ileocolic
Terminal ileum telescopes into the colon.

4.N5. a — Abdominal ultrasound (target sign)
Ultrasound shows target/doughnut sign.

4.N6. b — 6 months to 2 years
Most cases occur between 3 months and 3 years.

5.1. d — Oral feeding

5.2. a — Neonates

5.3. c — Colostomy, anoplasty, closure of colostomy

5.4. b — Intravenous line

5.5. c — Hyperactive baby

5.6. c — Tracheo-esophageal fistula

5.7. a — NGT/Ryle

5.8. b — No meconium passage and abdominal distention

5.9. b — Cross-table lateral X-ray

5.10. b — Rectourethral fistula

5.11. b — Double bubble sign

5.12. c — Nasogastric decompression

5.13. ✎ — Hirschsprung's disease, meconium ileus, duodenal atresia

5.14. ✎ — Hirschsprung's disease

5.15. ✎ — Duodenal atresia

5.16. b — False

5.17. b — False

5.18. a — True
المقصود إزالة الجزء الـ «removal of ganglionic» — الملف كاتب aganglionic.

5.19. a — True

5.20. b — False

5.21. a — True

5.22. a — True

5.23. ✎ — Anoplasty

5.24. ✎ — Colostomy then anoplasty (then closure of colostomy)

5.25. ✎ — Imperforate anus

5.26. ✎ — Insert NGT (Ryle)

5.N1. b — Colostomy, then anoplasty (PSARP), then closure of colostomy
High lesions need a staged approach; low lesions are treated by anoplasty.

5.N2. b — The first rectal temperature cannot be taken or no meconium passes in 24 h
Inspection of the perineum and failure to pass meconium are the earliest clues.

5.N3. b — Clean skin around stoma, use a well-fitting bag and watch stoma color
The stoma should be pink and moist; protect peristomal skin and empty the bag when 1/3 full.

5.N4. b — 24 hours
Allows bowel gas to reach the rectal pouch.

5.N5. a — Rectovestibular fistula
Males: rectourethral (bulbar) fistula.

5.N6. a — VACTERL
Screen for spinal, cardiac, renal anomalies.